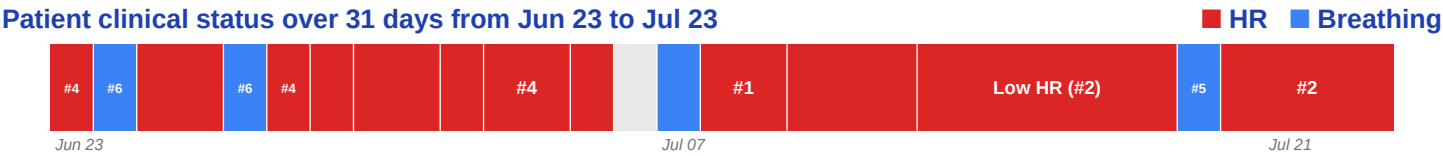


CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: JBPeriod: June 23 to July 23, 2024Report Date: July 24, 2024Coverage: Bed: 609/744h (81.9%)
Chair: 502/744h (67.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
115	183h	High Heart Rate	4	100%

Major Findings: Sustained high HR (avg 69, peak 131)

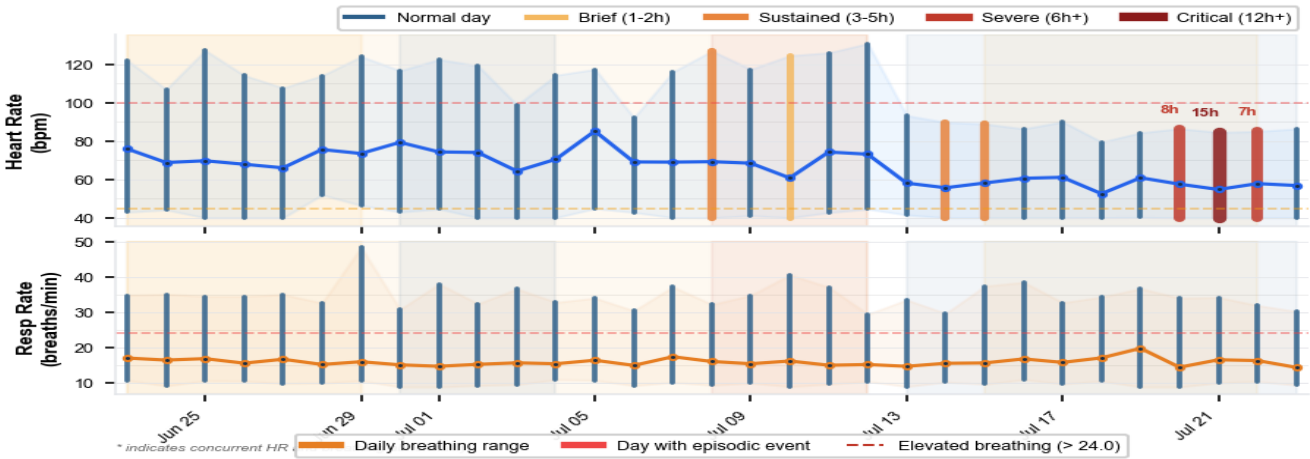
#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	High Heart Rate	127 bpm	7h	2h	1	69 bpm	Jul 08 to Jul 12
2	Low Heart Rate	40 bpm	61h	8h	2	58 bpm	Jul 13 to Jul 23
3	Low Heart Rate	40 bpm	7h	1h	1	73 bpm	Jun 30 to Jul 04
4	Elevated Heart Rate	131 bpm	77h	5h	3	72 bpm	Jun 23 to Jul 12
5	Elevated Breathing	37 brpm	21h	4h	2	16 brpm	Jul 15 to Jul 22
6	Elevated Breathing	48 brpm	10h	2h	1	16 brpm	Jun 23 to Jun 29
7	Very High Heart Rate (brief)	130 bpm	6h	2h	<1	122 bpm	Jun 29 to Jul 12

+ 1 additional condition(s) across High Breathing (brief); details in monitoring data.

The P5 to P95 heart rate spread was 52 bpm (44 to 96), indicating significant cardiac variability.

Clinical Pattern Observations:

- 1. Sustained finding: 8h continuous Low Heart Rate beginning Jul 20.
- 2. High variability: HR spread 52 bpm (43 to 95).
- 3. Clustered pattern: Episodic events on 7 of 31 days (22%).



Red bars indicate days with detected episodic events.
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Low HR < 45 bpm

Elevated HR > 95 bpm

High HR > 100 bpm

Very High HR > 110 bpm

Elevated Breathing > 24 brpm

High Breathing > 30 brpm

Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.